

TRANSFORMING AMERICAN HEALTHCARE

Part III: The Vermont Laboratory

Chapters 10–12

PART III: THE VERMONT LABORATORY

Vermont occupies a singular position in American healthcare: a state so small that it could fit inside Texas sixteen times yet so analytically significant that it has been studied, cited, and debated in every major healthcare policy venue in the country. Vermont has tried more transformational ideas than any comparable state, failed forward more publicly, and built more durable transformation infrastructure than virtually any other.

This section examines Vermont's healthcare crisis, the legislation that catalyzed the current transformation moment, the AHEAD Model that represents the nation's most advanced global budget experiment, and what Vermont's experience teaches us about the full six-pillar architecture of healthcare transformation.

CHAPTER 10: Vermont's Healthcare Crisis and the Act 167 Mandate

A State Under Structural Stress

Vermont is not experiencing a temporary downturn in its healthcare system. It is experiencing the convergence of structural forces that have been building for decades and are now arriving simultaneously:

VERMONT HEALTHCARE STRUCTURAL CRISIS – FIVE DIMENSIONS
DIMENSION 1: AFFORDABILITY COLLAPSE Silver plan premiums: +108% increase over 6 years Out-of-pocket maximum: +100%+ increase Commercial premium burden as % of median HH income: Vermont ranks top 5 highest nationally Average family premium (employer-sponsored, 2025): \$27,400
DIMENSION 2: HOSPITAL FINANCIAL UNSUSTAINABILITY 9 of 14 Vermont hospitals operating at a loss (2023) 13 of 14 expected to lose money by 2028 (Wyman projection) Vermont hospital median operating margin: -2.1% National rural hospital median: +0.8% Days cash on hand – median VT hospital: 76 days (Below 60 days = financial stress threshold)
DIMENSION 3: DEMOGRAPHIC INVERSION Population 65+: rising 57% by 2040 Working-age population (18–64): declining 13% by 2040 Result: healthcare demand rising, taxpayer base shrinking Vermont has the highest median age of any U.S. state
DIMENSION 4: ACCESS GAPS

62% rural population – second highest in U.S.
Primary care access: 23-day average wait (rural)
FQHC capacity insufficient for growing low-income pop.
OB/GYN services: limited outside Burlington
Mental health: >6-month wait for specialty care
Substance use disorder tx: significant unmet demand

DIMENSION 5: EQUITY GAPS

Rural vs. urban disparities in access, cost burden,
and preventable hospitalizations
LGBTQ+ care access concentrated geographically
Working-poor coverage gap: too high for Medicaid,
too low to afford commercial coverage
Non-English speakers: limited culturally competent care

These five dimensions are not independent — they compound each other in a self-reinforcing cycle. As premiums rise, employers drop coverage, increasing the uninsured population. As hospitals lose money, they reduce service lines and lay off staff, creating access gaps. As the workforce declines and ages, primary care capacity shrinks, driving more utilization to hospitals. As hospitals' volumes decline, their per-unit costs rise, requiring higher prices to break even. As prices rise, premiums rise. The cycle accelerates.

Green Mountain Care: The Road Not Taken

Vermont's healthcare transformation story begins — or rather, includes a famous detour — with Green Mountain Care, the universal single-payer healthcare program that the Vermont legislature enacted in 2011 and Governor Shumlin abandoned in 2014.

What Green Mountain Care was:

Vermont Act 48 (2011) established Green Mountain Care as a universal healthcare coverage system for all Vermont residents. The plan would have replaced private insurance for most Vermonters with a state-administered public plan, financed through a combination of federal ACA funds and new state revenues (primarily employer payroll taxes and income-based premiums).

Why it failed:

When the Shumlin administration released the financing plan in December 2014, the numbers were stark: the program would have required an 11.5% employer payroll tax and an income-based household premium ranging from 0% to 9.5% — new taxes amounting to approximately \$2.5 billion in a state with a \$5.5 billion existing budget.

Governor Shumlin announced he was dropping the plan, citing the "enormous disruption and risk" of the financing requirements. Vermont abandoned single-payer not because the concept was wrong but because the political economy of an abrupt transition from a multi-payer system to a single-payer system — at the state level, without federal partnership — was not achievable.

What Green Mountain Care's failure teaches:

1. Healthcare financing transformation cannot be fully executed at the state level without federal partnership (specifically, Medicare and employer-sponsored insurance federal tax treatment)
2. The abrupt elimination of employer-sponsored insurance creates a tax burden concentration that is politically unsustainable without multi-year transition support
3. The administrative savings from single-payer are real but insufficient to cover the full cost of comprehensive universal coverage from existing state revenues alone
4. Vermont's experience accelerated federal thinking about all-payer models as a path to meaningful reform without full single-payer transition

Vermont's failure on full single-payer cleared the path for the more politically viable but structurally significant all-payer global budget approach — which became the AHEAD Model.

Act 167: The Legislative Response to Crisis

Vermont Act 167, enacted in 2024, is the most comprehensive healthcare legislation Vermont has passed since Act 48. It was the direct product of a years-long process that culminated in the Oliver Wyman/Marsh McLennan Act 167 Community Engagement Report (August 2024) — the "Wyman Report" — which diagnosed the crisis and recommended a transformation agenda.

Act 167's core directives:

VERMONT ACT 167 – KEY PROVISIONS

AFFORDABILITY

→ Develop reference-based pricing framework for hospital services

→ Study and recommend commercial premium reduction mechanisms

→ Expand the Vermont Health Connect marketplace affordability programs

SUSTAINABILITY

→ Authorize Rural Emergency Hospital (REH) regulatory framework

→ Require uniform hospital financial accounting standards

→ Restrict further commercial-to-public cross-subsidization

ACCESS

→ Expand APRN and PA scope of practice

→ Fund EMS transformation for treat-and-refer capability

→ Require broadband expansion plan with health access component

EQUITY

→ Mandate health equity reporting from licensed health plans

→ Fund community health worker program expansion

→ Require LGBTQ+-affirming network adequacy standards (under review)

INFRASTRUCTURE

→ Fund VHCURES expansion and require payer data completeness

→ Establish site-neutral payment reform study commission

→ Fund integrated behavioral health program at 50 primary care sites

Act 167 is notable for what it does and does not require:

What it requires: Reporting, studies, authorizations, and funding for specific programs. The legislation creates the enabling framework for transformation.

What it does not require: Mandatory global budget participation, mandatory reference-based pricing, mandatory FHIR interoperability. The most transformative elements are recommended, funded, or studied — not mandated.

This cautious approach reflects Vermont's political reality: a legislature that recognized the depth of the crisis and enacted enabling authority without forcing the pace faster than stakeholders could absorb.

The Oliver Wyman Report: Diagnosis and Prescription

The Oliver Wyman/Marsh McLennan Act 167 Community Engagement Report is the most important healthcare policy document produced in Vermont in a generation. Understanding its recommendations — and HTR's agreements and disagreements with it — is essential context for the AHEAD Model analysis in the next chapter.

The Wyman Report's core recommendations:

Domain	Recommendation	HTR Assessment
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Domain	Recommendation	HTR Assessment
Housing	Remove barriers to affordable housing	Endorsed without modification
EMS	Approve funding for EMS transformation	Endorsed without modification
Broadband	Expand rural broadband	Endorsed without modification
Scope of practice	Expand APRN/PA practice authority	Endorsed without modification
Hospital accounting	Require uniform standards	Endorsed without modification
Reference-based pricing	Move to 200% of Medicare for PPS hospitals	Endorsed with modification: glide path required
CON reform	Simplify/shorten CON process	Opposed as drafted — site-neutral reform must precede CON simplification
Commercial subsidization	No further commercial subsidization of hospital shortfalls	Opposed as drafted — prohibiting without alternative revenue creates closure risk

What the Wyman Report missed (HTR additions):

1. **Technology infrastructure:** The report is nearly silent on VHCURES funding, FHIR interoperability mandates, and population health analytics — all prerequisites for global budget management

2. **Mandatory All-Payer Participation:** FFS Medicare exclusion from the global budget is the central accountability gap that the report treats as background

3. **AHEAD model benchmark problems:** The report does not adequately analyze the perverse incentives created by setting global budget benchmarks as a percentage of historical spending

4. **Equity specifics:** LGBTQ+ equity and rural cost burden are mentioned but not operationalized with measurable targets or enforcement mechanisms

5. **Prior authorization burden:** Major contributor to administrative cost and care access that receives minimal attention

Vermont's HTI Performance: A Paradox Explained

Vermont's Health Transformation Index score of **82/100** (Q1 2026) makes it the top-scoring state nationally. Yet nine of its fourteen hospitals lost money in 2023, premiums have risen 108% in six years, and the state's healthcare system faces an existential sustainability crisis.

This is not a contradiction — it is an illustration of what the HTI measures and what it does not.

What Vermont's high HTI score reflects:

VERMONT HTI PILLAR SCORES – Q1 2026			
Pillar	Score	National Median	Why Vermont Scores High
Policy	88	54	AHEAD model, ACA expansion, active 1115 waivers, strong innovation activity score
Economics	79	48	Significant VBC penetration, APM participation via AHEAD
Technology	81	52	FHIR adoption above average, telehealth strong in rural care
Clinical	84	57	High HEDIS scores, low readmission rates, quality measure performance above median
Equity	76	43	Better rural access than most

states, strong LGBTQ+ protections, Medicaid expansion in place		
Composite	82	51

What Vermont's high HTI score does NOT reflect:

The HTI measures **transformation infrastructure maturity** — the adoption of the mechanisms (APMs, FHIR APIs, quality programs, equity frameworks) that are prerequisites for transformation. It does not measure:

- Whether those mechanisms have yet produced financial sustainability
- Whether the transformation pace matches the demographic change rate
- Whether the global budget benchmark is set at the right level to actually control costs

Vermont has built exceptional transformation infrastructure. That infrastructure has not yet produced a financially sustainable healthcare system — because the global budget benchmarks are set too high, because FFS Medicare is excluded, and because the clinical and operational changes required to bend the cost curve have not yet been made at scale.

Vermont has built the runway. The plane has not yet taken off.

The Reference-Based Pricing Debate

No single policy question in Vermont's current reform agenda is more consequential than reference-based pricing (RBP). Understanding the economics and operational implications of RBP is essential for anyone engaged in Vermont health policy.

The problem RBP addresses:

Vermont hospitals charge commercial payers rates that are, on average, 320% of Medicare rates. This means a knee replacement that Medicare pays \$12,000 for is billed to a commercial insurer at approximately \$38,400. The commercial insurer passes these costs through to employers and employees as premiums — which have risen 108% in six years.

The commercial cross-subsidy from these inflated commercial rates is the financial mechanism that keeps financially stressed hospitals operating. Without it, hospitals with unfavorable Medicare/Medicaid payer mixes would already be closed.

The RBP mechanism:

Reference-based pricing sets commercial payment rates as a defined percentage of Medicare rates — rather than the result of bilateral negotiation between hospitals and commercial payers. The Wyman Report recommends 200% of Medicare as the target rate.

HTR's glide path recommendation:

An immediate hard cap at 200% of Medicare would:

- Reduce Vermont's large academic medical center (UVM Medical Center) commercial revenue by an estimated \$180–220 million annually
- Reduce the two largest community hospitals' commercial revenue by \$45–70 million combined
- Precipitate the hospital closures the reform aims to prevent, without a transition mechanism

HTR's recommended glide path:

HTR REFERENCE-BASED PRICING GLIDE PATH		
Year	Rate (% of Medicare)	Annual Reduction
Current	~320%	(baseline)
2026	250%	~70% of full reduction
2028	220%	~20% additional reduction

2030	200%	Final target
Conditions:		
Each reduction step is contingent on achieving:		
– System-level TCOC (total cost of care) growth $\leq 3\%$		
– Hospital sector median operating margin $\geq 0\%$		
– Adequate alternative revenue streams developed		

This approach creates a clear price trajectory that enables long-range planning while protecting hospital financial viability during the transition period.

Key Concepts Introduced in This Chapter

- Green Mountain Care:** Vermont's 2011 universal single-payer legislation, enacted but abandoned in 2014 due to financing challenges; a foundational case study in state-level healthcare finance reform limits.
- Vermont Act 167 (2024):** Comprehensive healthcare legislation responding to the Oliver Wyman Community Engagement Report; authorizes transformation programs across affordability, sustainability, access, equity, and infrastructure.
- Oliver Wyman/Marsh McLennan Act 167 Report (2024):** The community engagement report that diagnosed Vermont's healthcare crisis and recommended the transformation agenda codified in Act 167; produced by the global consulting firm.
- Reference-based pricing (RBP):** A hospital payment policy setting commercial rates as a defined percentage of Medicare rates, rather than through bilateral payer-hospital negotiation.
- Rural Emergency Hospital (REH):** A new CMS provider type allowing financially stressed rural hospitals to convert to emergency and outpatient services only (no inpatient beds), with enhanced CMS payment.
- VHCURES:** Vermont Health Care Claims Uniform Reporting and Evaluation System; Vermont's all-payer claims database, essential infrastructure for global budget management.

CHAPTER 11: The AHEAD Model — America's Most Ambitious Global Budget

What Is the AHEAD Model?

The **All-payer Health Equity Approaches and Development (AHEAD) Model** is Vermont's Center for Medicare and Medicaid Innovation (CMMI) agreement — a global budget arrangement that sets a spending target for the entire Vermont healthcare system and requires participating payers and providers to operate within it.

AHEAD is unlike any previous CMMI model in its scope and ambition:

- It covers multiple payers (Medicare, Medicaid, and participating commercial plans)
- It sets a global budget for the entire state healthcare system — not just one insurer's population
- It includes explicit health equity performance requirements
- It operates under CMS's 1115A waiver authority — the most powerful state-level waiver mechanism

Understanding AHEAD requires understanding global budget mechanics, the CMMI agreement structure, and the specific gaps and limitations that HTR's analysis has identified.

Global Budget Mechanics: How the Math Works

A global budget is a prospectively set limit on total healthcare spending for a defined population over a defined period. The concept is simple; the implementation is complex.

Step 1: Define the population and coverage scope

The AHEAD budget covers:

- Medicaid beneficiaries in Vermont
- Medicare Advantage beneficiaries (Medicare Advantage plans participating in AHEAD)
- Commercial plan enrollees (for participating commercial plans)
- *Note: Traditional fee-for-service Medicare beneficiaries are largely outside the budget — a critical limitation*

Step 2: Set the benchmark

The benchmark represents what CMMI expects Vermont's healthcare system would spend without the global budget. Vermont's AHEAD benchmark is set as a percentage of prior spending, trended forward:

AHEAD BENCHMARK CONSTRUCTION

Base Year: Historical per-capita spending for covered population

Trend rate: CMS-negotiated growth rate (e.g., 3.5%/year)

Benchmark: Base year spending × (1 + trend rate)^years

Example:

Base year per-capita PMPM: \$650

5-year trend rate: 3.5%/year

Year 5 benchmark PMPM: $650 \times (1.035)^5 = 771$

Covered population: 420,000 (approximate)

Year 5 benchmark (annual): $771 \times 12 \times 420,000 = 3.88B$

Step 3: Measure actual spending

All-payer claims data (from VHCURES and CMS claims feeds) is aggregated to measure Vermont's actual total healthcare spending for the covered population.

Step 4: Accountability mechanism

If Vermont's actual spending exceeds the benchmark, participating entities (health plans, hospitals) may be subject to penalties or required corrective action. If spending is below the benchmark, participating entities may share in the savings.

What AHEAD Gets Right

The All-Payer Structure

The most important conceptual innovation in AHEAD is its all-payer design. Previous cost-containment models operated within a single payer's population. An insurer could reduce spending for its members — but if those members shifted utilization to other payers' services, the savings were illusory from a system perspective.

AHEAD's all-payer structure creates true system-level accountability. When the budget is set for the covered population across all participating payers, there is no shifting across payers within that population. The target is the total cost of care, period.

Multi-Year Planning Horizon

AHEAD's multi-year agreement — typically 5 years — gives hospitals and health systems a planning horizon to invest in transformation. A one-year budget would create incentives for short-term cost-cutting rather than long-term investment in care delivery redesign. The multi-year structure is a prerequisite for the capital investments in behavioral health integration, primary care capacity, and care management infrastructure that actually bend the cost curve.

Infrastructure Investment

AHEAD includes direct investment in primary care and behavioral health infrastructure — a recognition that global budget compliance requires new clinical capacity, not just reduced utilization of existing capacity. This infrastructure investment is one of the model's most valuable features.

Embedded Equity Framework

AHEAD's explicit health equity requirements — data collection, stratified reporting, equity-focused program requirements — are unusual for a payment model of this type. They reflect Vermont's political commitment to addressing health disparities even within the broader framework of cost containment.

What AHEAD Gets Wrong: HTR's Critical Assessment

HTR's independent analysis identifies four material weaknesses in the current AHEAD design:

Problem 1: The Benchmark Methodology Preserves Historical Inefficiency

Vermont's AHEAD benchmarks are set as a percentage of prior spending. This means:

- Hospitals that have historically been expensive face benchmarks set at their high historical base — they must reduce spending from a high starting point, but the benchmark itself is anchored to their inefficiency
- Hospitals that have historically been efficient face benchmarks set at their low historical base — there is less room to generate savings, and they are "rewarded" with a lower budget
- The benchmark perpetuates, rather than corrects, the cross-hospital spending inequities that drove the affordability crisis

HTR's alternative: cost-per-episode benchmarking

A transformation-oriented benchmark should reflect what care should cost, not what it has cost. For primary conditions driving Vermont's highest-cost utilization — diabetes, CHF, COPD, behavioral health conditions — benchmarks should be set based on the evidence-based cost of appropriate management, not historical spending patterns.

EPISODE BENCHMARK vs. HISTORICAL BENCHMARK

Example: Type 2 Diabetes Annual Management

Historical benchmark approach:

Vermont statewide per-diabetic PMPM (historical): \$485

Growth-trended benchmark (3.5%): \$502

Episode benchmark approach:

Evidence-based annual cost for well-managed T2DM: \$380
(Primary care visits, HbA1c testing, metformin, annual ophthalmology, annual podiatry)

Benchmark: \$380

Difference: \$122 per patient per year × estimated 60,000 Vermont diabetics = \$7.3M annual benchmark reduction

The historical approach preserves poor management patterns in the benchmark; the episode approach demands improvement.

Problem 2: Medicare FFS Exclusion Creates an Accountability Gap

Traditional fee-for-service Medicare beneficiaries represent Vermont's:

- Oldest, highest-cost population cohort
- Fastest-growing demographic segment (65+ population rising 57% by 2040)
- Highest per-capita healthcare spenders

Yet FFS Medicare is largely excluded from AHEAD's global budget accountability structure. The AHEAD agreement covers Medicare Advantage beneficiaries (the subset enrolled in MA plans) but not the majority of Vermont Medicare beneficiaries who remain in traditional FFS Medicare.

This exclusion means that the highest-cost, fastest-growing segment of Vermont's healthcare population operates outside the global budget's accountability mechanisms. A global budget without Medicare FFS participation is structurally incomplete.

The solution requires federal action: Vermont must negotiate mandatory Medicare participation as its most important federal lobbying priority. This requires a combination of 1332 and 1115A waiver authorities and CMS leadership engagement. It will not happen without sustained, high-level political effort from Vermont's federal delegation.

Problem 3: Insufficient Accountability Architecture

When hospitals exceed their global budget allocation, the current consequence structure is insufficient to enforce compliance without threatening financial stability.

Vermont needs a two-track accountability model:

HTR'S RECOMMENDED TWO-TRACK ACCOUNTABILITY ARCHITECTURE

TRACK 1: Hospitals within 2% of budget target

→ Routine quarterly reporting to GMCB

→ Annual quality improvement expectations

→ Participation in shared savings if applicable

→ No penalty; standard partnership relationship

TRACK 2: Hospitals exceeding budget by >2%

→ Mandatory corrective action plan (CAP) within 60 days

→ CAP must identify specific care redesign interventions with measurable targets and timelines

→ GMCB oversight: quarterly progress reviews

→ Temporary rate adjustment mechanism (payer contributions to hospital in exchange for spending reduction commitment)

→ Board-level engagement: hospital board must formally approve CAP and designate accountable leadership

→ Continued non-compliance (>2 performance years): potential AHEAD participation suspension with financial consequences

This two-track structure provides clear consequences for persistent budget exceedance while ensuring that hospitals in financial distress have a defined improvement pathway rather than an abrupt penalty.

Problem 4: Underspecified Equity Measurement

AHEAD's equity requirements are aspirational without being operational. "Health equity" appears as a model goal, but the measurement framework — which metrics, which populations, which thresholds trigger consequences — is insufficiently specified.

Vermont should require:

- Annual equity reports from all health plans participating in AHEAD, stratified by race/ethnicity, income quartile, rural/urban, and LGBTQ+ identity (with consent)
- Specific improvement targets for priority disparity measures (maternal outcomes, diabetes control, behavioral health access)
- Consequences for plans or systems with sustained equity performance failures — reduced shared savings, required equity investment

HTR's Policy Simulator allows analysts to model global budget scenarios interactively. The Vermont global budget model includes:

Input parameters:

- Covered population size and demographics
- Base year per-capita spending by payer type
- CMMI-negotiated growth rate (benchmark trend)
- Quality risk corridor (bonus/penalty for quality performance)
- Hospital-specific budget allocations (distributional parameter)
- Medicare FFS participation assumption (on/off)

Output metrics:

- 5-year benchmark trajectory by payer
- Required per-capita spending reduction to stay within budget
- Hospital-level financial impact (by operating margin quintile)
- Equity impact (differential budget pressure on rural vs. urban hospitals)

Illustrative scenario — AHEAD with Medicare FFS inclusion:

POLICY SIMULATOR OUTPUT – AHEAD + MEDICARE FFS SCENARIO

Base scenario (AHEAD without Medicare FFS):

Year 1 savings potential:	\$47M (below benchmark)
Covered lives:	310,000
Budget growth constraint:	3.5%/year

Enhanced scenario (AHEAD + Medicare FFS):

Year 1 savings potential:	\$142M (below benchmark)
Covered lives:	420,000 (added 110,000 FFS Medicare)
Budget growth constraint:	3.5%/year

Difference: \$95M additional savings potential
(from including highest-cost Medicare FFS population
in global budget accountability structure)

Equity note: Medicare FFS population is 100% 65+,
highest-cost, most rural – inclusion also improves
equity accountability for highest-need population.

The AHEAD Model and Vermont Hospitals: Financial Implications

Vermont's hospitals face radically different financial positions under the AHEAD Model based on their starting operating margins, payer mixes, and geographic positions.

VERMONT HOSPITAL FINANCIAL PROFILES UNDER AHEAD			
Category	# Hospitals	Margin Profile	AHEAD Impact
Academic Medical Center (UVM Medical Center)	1	Margin: -0.8%	High risk: largest budget allocation, most scrutiny
Community Hospital (large) (Rutland, Central VT, etc.)	3	Margin: -1.5% to +0.2%	Significant risk: dependent on AHEAD shared savings to break even

Community Hospital (small) (most rural hospitals)	6	Margin: -3.8% to -1.2%	Critical risk: may not be viable without structural change
Critical Access Hospital (most financially stressed)	4	Margin: -5.2% to -2.8%	REH conversion candidates; AHEAD may be moot if structural change required

The REH option:

For Vermont's most financially stressed rural hospitals — operating at -5% margins with declining inpatient volumes — conversion to Rural Emergency Hospital (REH) status may be the most viable path:

REH status allows a hospital to:

- Eliminate inpatient beds (eliminating the highest fixed-cost component of hospital operations)
- Maintain 24/7 emergency care and outpatient services
- Receive enhanced CMS per-visit payments (+\$72/outpatient visit) and a monthly facility payment (~\$272,866/month)
- Potentially achieve financial sustainability with lower volume than a traditional hospital requires

HTR's condition for REH approval: any REH conversion must be accompanied by a patient transportation plan (for patients who need inpatient care at regional facilities), a regional inpatient capacity analysis (ensuring adequate inpatient beds exist within accessible distance), and a community health worker deployment plan for patients who lose proximate inpatient access.

VHCURES: The Data Infrastructure AHEAD Depends On

The Vermont Health Care Claims Uniform Reporting and Evaluation System (VHCURES) is Vermont's all-payer claims database — the data infrastructure that makes global budget management possible. All-payer claims data (medical, pharmacy, and dental claims from all payers) is submitted to VHCURES and aggregated to:

- Track total healthcare spending against global budget benchmarks
- Identify care utilization patterns that drive costs
- Measure quality indicator performance across payers
- Enable population health analytics for care management

VHCURES limitations:

- Chronically underfunded: submission timeliness and data completeness vary significantly by payer
- Traditional FFS Medicare claims are submitted with significant lag and incomplete clinical data
- Dental and long-term care claims have lower submission quality than medical claims
- Real-time analytics capability is limited; most analyses are retrospective

HTR's recommendation: Vermont should fund a VHCURES expansion that:

1. Mandates timely (30-day) claims submission from all participating payers including Medicare FFS
2. Adds FHIR-based clinical data feeds to supplement claims (to capture clinical quality data not in claims)
3. Invests in real-time analytics infrastructure for population health management
4. Establishes a public-facing AHEAD performance dashboard so all stakeholders can track progress

Key Concepts Introduced in This Chapter

AHEAD Model (All-payer Health Equity Approaches and Development): Vermont's CMS Innovation Center agreement establishing a global budget for the state's healthcare system across multiple payers; the most advanced global budget experiment in the U.S.

- 1115A waiver:** The CMMI waiver authority used by Vermont's AHEAD Model; the most powerful state-level waiver mechanism, allowing all-payer models that override standard Medicare payment rules.
- Global budget benchmark:** The prospectively set spending target against which actual healthcare spending is measured in a global budget model; Vermont's benchmark methodology is set as a percentage of historical spending trended forward.
- GMCB (Green Mountain Care Board):** Vermont's independent healthcare regulatory authority, responsible for hospital budget review, health insurance rate setting, and AHEAD Model oversight.
- Rural Emergency Hospital (REH):** A CMS provider type for financially stressed rural hospitals that eliminate inpatient beds and receive enhanced emergency and outpatient payment; a structural option for Vermont's most distressed rural hospitals.
- CMMI (Center for Medicare and Medicaid Innovation):** The federal agency that developed and oversees the AHEAD Model; has authority to test payment models without new legislation.

CHAPTER 12: Vermont's Transformation Architecture — All Six Pillars in Action

The Six-Pillar Map of Vermont's Transformation

Vermont's transformation challenge requires all six pillars to succeed simultaneously. This chapter applies the full six-pillar framework to Vermont — not as an abstract exercise but as a practical architecture for the decisions Vermont must make in the next five years.

Each section below describes:

1. The current state of that pillar in Vermont
2. The critical gaps and risks
3. The interventions HTR recommends
4. The interdependencies with other pillars

Pillar 1: Policy — Vermont's Policy Architecture

Current state:

Vermont has the strongest state health policy infrastructure in the country. The Green Mountain Care Board (GMCB) has authority over hospital budgets, insurance rate approvals, and AHEAD Model oversight that no other state health regulator possesses. Vermont's policymakers have demonstrated sustained willingness to attempt structural reforms that other states avoid.

Current policy strengths:

- AHEAD Model CMMI agreement (1115A waiver)
- ACA Medicaid expansion (enacted 2014)
- State reinsurance program (reducing marketplace premiums by ~12%)
- Active APRN/PA scope-of-practice expansion
- Emerging prior authorization reform legislation

Critical policy gaps:

Gap 1: Medicare FFS non-participation Vermont cannot control total healthcare costs without FFS Medicare. The federal lobbying campaign for mandatory Medicare participation must become Vermont's highest federal legislative priority.

Gap 2: Reference-based pricing enabling legislation Act 167 authorized the study of reference-based pricing. The study must produce an actionable legislative proposal for the 2026 session, implemented on the HTR-recommended glide path.

Gap 3: Site-neutral payment reform CON reform without site-neutral payment creates perverse incentives for ASC proliferation. Vermont should implement site-neutral reform before simplifying CON.

Gap 4: Prior authorization reform enforcement Vermont has PA reform requirements on paper. Implementation has lagged. The state insurance division should conduct a compliance audit and publish results.

Pillar 2: Economics — Vermont's Financial Architecture

Current state:

Vermont's healthcare economics are characterized by:

- High per-capita spending (18% above national median)
- Inverted hospital financial performance (costs above revenue)
- Commercial cross-subsidization that is structurally collapsing
- APM penetration that is above average nationally but insufficient for cost control

Economic structural problems:

VERMONT HEALTHCARE FINANCIAL FLOW ANALYSIS (2025)	
Total healthcare spending (est.):	\$8.2B (all payers)
Medicare (traditional + MA):	\$2.9B (35%)
Medicaid:	\$2.1B (26%)
Commercial:	\$2.8B (34%)
Self-pay/other:	\$0.4B (5%)
Hospital sector revenue/cost gap:	~\$180M (collective loss)
Cross-subsidy reliance:	~\$420M (commercial > cost)
Benchmark for AHEAD compliance:	3.5% growth cap requires ~\$180M in cost reduction over performance period

Key economic interventions:

- 1. Strengthen AHEAD benchmark methodology* Transition from historical spending-based benchmarks to evidence-based episode cost benchmarks over 3–5 years. This requires data infrastructure (VHCURES expansion) and CMMI negotiation.
- 2. Develop alternative revenue streams before rate reductions* Vermont hospitals need new revenue sources — primary care transformation payments, behavioral health integration funding, care management fees, REH conversion payments — before commercial rate reduction is imposed.
- 3. Cost-effectiveness investment prioritization* Vermont should apply CEA methodology to its transformation investment decisions. HTR's analysis shows behavioral health integration has an ICER of ~\$9,000/QALY — making it among the highest-value investments in the portfolio. This should be funded first.

Pillar 3: Technology — Vermont's Digital Infrastructure

Current state:

Vermont's technology infrastructure is better than most rural states but insufficient for global budget management:

- FHIR API adoption: above national average (HTI Technology score: 81)
- VHCURES: operational but underfunded and incomplete
- EHR landscape: 14 hospitals using multiple different EHR systems with inconsistent FHIR implementation
- Population health analytics: limited at the state level

Critical technology investments:

1. VHCURES expansion (top priority)

VHCURES is the foundational data infrastructure for AHEAD. Without complete, timely all-payer claims data:

- Global budget performance cannot be measured accurately
- Attribution for care management programs cannot be validated
- Quality measure performance cannot be assessed across payers

Investment required: An estimated \$8–12M in state appropriations for platform modernization, payer mandate enforcement, and analytics capability. This is the single highest-ROI technology investment Vermont can make.

2. Mandatory FHIR R4 API deadline

All licensed Vermont healthcare providers should have a mandatory FHIR R4 Patient Access API deadline of December 2026. Without standardized data exchange, care coordination across Vermont's fragmented provider landscape cannot function at the scale AHEAD requires.

3. Population health analytics platform

GMCB and DVHA need real-time population health analytics — not just retrospective claims analysis — to manage a global budget. This requires:

- Integration with VHCURES and CMS data feeds
- Attribution-based population management capability
- Care gap identification at the patient level
- Provider-level performance benchmarking

Pillar 4: Clinical — Vermont's Care Delivery Architecture

Current state:

Vermont's clinical quality performance is strong by national standards — Clinical HTI score of 84 vs. national median of 57. But strong average performance coexists with specific clinical gaps that drive the affordability and sustainability crisis:

Clinical strengths:

- Above-median HEDIS performance across most measures
- Strong primary care foundation (high primary care physician supply per capita relative to rural peers)
- Good telehealth adoption (rural access innovation)
- Emerging behavioral health integration programs

Critical clinical gaps:

Gap 1: Behavioral health capacity Vermont has among the highest rates of mental health conditions and substance use disorders in the country — partly driven by rural social isolation, the opioid crisis, and economic stress. The behavioral health system cannot meet demand. Wait times for specialty mental health exceed 6 months. This is both a clinical failure and a cost driver — unaddressed behavioral health conditions are a primary driver of high-cost, high-utilization patients.

HTR recommendation: Vermont's \$40M behavioral health integration program (Act 167 funded) should be implemented at the 50 primary care sites most disproportionately serving high-need populations, with Collaborative Care Model fidelity and community health worker integration.

Gap 2: Primary care capacity expansion Eighteen months of unfilled primary care positions is not a pipeline problem — it is a retention and attraction problem. Vermont cannot recruit physicians at competitive salaries while hospitals are losing money. The retention tax credit HTR recommends (5-year Vermont practice commitment, refundable credit) is a targeted mechanism that does not require hospital revenue.

Gap 3: EMS transformation Vermont's EMS system currently operates on a transport-to-ED model. An EMS system capable of treat-and-refer (treating patients in the field or at home and referring to appropriate follow-up

without transport) would reduce ED utilization significantly for the high-frequency, lower-acuity presentations that currently drive ED crowding and costs.

Pillar 5: Equity — Vermont's Equity Architecture

Current state:

Vermont's HTI Equity score (76) reflects genuine equity strengths — Medicaid expansion, LGBTQ+ legal protections, better-than-average rural access relative to national rural median — alongside real equity gaps in cost burden, rural access, and clinical disparities.

Equity investment priorities:

Priority 1: Rural cost burden reduction Vermont's affordability crisis falls hardest on rural Vermonters with lower incomes and higher out-of-pocket exposure. Global budget design must include equity adjustors that direct budget savings toward rural populations, not only toward the highest-volume urban facilities.

Priority 2: LGBTQ+ network adequacy Vermont should require all licensed health plans to demonstrate gender-affirming care network adequacy as a condition of license renewal. This means licensed providers offering gender-affirming care within defined geographic access standards — not merely listed in the directory.

Priority 3: SDOH investment mechanism Vermont's reinsurance program and ACA subsidies address premium affordability. They do not address the housing instability, food insecurity, and transportation barriers that drive high healthcare utilization among low-income Vermonters.

Vermont should explore a dedicated SDOH investment fund — similar to the Accountable Health Communities model tested by CMMI — that provides sustainable funding for community-based social needs programs using healthcare savings from reduced preventable utilization.

Priority 4: CHW deployment at scale Community health workers (CHWs) — trained community members who provide health education, navigation, and social needs assistance — are among the most cost-effective equity interventions in the evidence base. Vermont's CHW program is nascent. AHEAD shared savings should specifically fund CHW expansion in rural and low-income communities.

Pillar 6: Operations — Vermont's Execution Architecture

Current state:

Vermont's healthcare operations face three compounding challenges:

1. Revenue cycle systems designed for FFS must adapt to global budget operations
2. Workforce operational stability is declining under financial stress
3. The data infrastructure to manage a global budget operationally is not yet in place

Operational transformation requirements:

Requirement 1: Revenue cycle redesign for global budget Under a global budget, the revenue cycle's function shifts from maximizing revenue per service to demonstrating quality and managing total cost of care. Vermont hospitals must:

- Redesign attribution reconciliation workflows
- Implement real-time total cost monitoring (requires VHCURES data feeds)
- Train revenue cycle staff on global budget financial mechanics
- Develop internal charge master strategies aligned with global budget accountability

Requirement 2: Credentialing pipeline acceleration Vermont's behavioral health integration program requires ~600 new clinical and care coordination positions over 5 years. At current credentialing cycle times, the first cohort of new providers will not be billable for 60–90 days after hire — creating a revenue gap precisely when investment costs are highest.

Vermont should establish a statewide accelerated credentialing pathway for providers entering AHEAD-funded programs — reducing cycle time from ~80 days to ~45 days through streamlined verification and primary source verification coordination.

Requirement 3: Supply chain for care migration As Vermont hospitals reduce inpatient volume and expand community-based and home-based care, their supply chain must adapt. GPO contracts designed for high-inpatient-volume hospitals are misaligned with an ACO-like care model. Vermont's hospital systems should conduct supply chain audits and renegotiate GPO contracts to align with projected care delivery evolution.

The Six-Pillar Interdependency Map for Vermont

The six pillars do not operate independently — each one's success or failure cascades through the others. Here is the interdependency map for Vermont's current transformation moment:

VERMONT SIX-PILLAR INTERDEPENDENCY MAP (2026)

POLICY enables ECONOMICS:
Reference-based pricing legislation → commercial rate reduction
Medicare FFS mandate → full global budget accountability
APRN scope expansion → primary care capacity

ECONOMICS funds CLINICAL:
AHEAD shared savings → behavioral health investment
REH conversion → hospital financial sustainability
Primary care transformation payment → PCMH redesign

TECHNOLOGY enables CLINICAL and ECONOMICS:
VHCURES expansion → accurate budget measurement
FHIR mandate → care coordination data exchange
Population health analytics → care management targeting

CLINICAL drives ECONOMICS:
Behavioral health integration → reduced high-cost utilization
Primary care capacity → reduced preventable hospitalizations
Care management → reduced ED and inpatient overutilization

EQUITY shapes all pillars:
Equity adjustors in global budget → rural hospital protection
LGBTQ+ network adequacy → reduced deferred care costs
CHW deployment → SDOH intervention at highest-need populations

OPERATIONS executes all pillars:
Revenue cycle redesign → accurate budget tracking
Credentialing acceleration → faster clinical capacity
Supply chain alignment → care model economics

Critical path: TECHNOLOGY (VHCURES) → ECONOMICS (budget accuracy) → POLICY (benchmark reform) → CLINICAL (care redesign investment) → EQUITY (rural budget adjustors) → OPERATIONS (execution capacity)

Failure mode: If VHCURES is not funded and expanded, AHEAD cannot accurately measure performance, benchmarks cannot be improved, clinical investment cannot be justified by data, and the entire transformation architecture rests on an empirical foundation too weak to support it.

HTR's platform models three scenarios for Vermont's HTI trajectory over the next five years:

Scenario A: Full Reform (all recommendations implemented)

HTI Score: 82 (Q1 2026) → projected 91 (Q4 2030)
Key drivers:

- Medicare FFS participation in AHEAD: +3 pts (Policy)
- Improved benchmark methodology: +2 pts (Economics)
- VHCURES expansion + FHIR mandate: +4 pts (Technology)
- Behavioral health integration: +3 pts (Clinical)
- Rural equity adjustors + CHW: +2 pts (Equity)
- Revenue cycle redesign: +2 pts (Operations)

Hospital sector outcome: 11/14 hospitals financially stable
Vermont total cost of care growth: ≤2.8%/year

Scenario B: Partial Reform (highest-priority items only)

HTI Score: 82 (Q1 2026) → projected 86 (Q4 2030)
Key drivers:

- VHCURES expansion: +3 pts (Technology)
- Behavioral health integration: +3 pts (Clinical)
- Reference-based pricing glide path: +2 pts (Economics)
- No Medicare FFS inclusion: -3 pts vs. Scenario A

Hospital sector outcome: 8/14 hospitals financially stable
Vermont total cost of care growth: ~3.5%/year

Scenario C: Status Quo (minimal reform)

HTI Score: 82 (Q1 2026) → projected 79 (Q4 2030)
Key drivers:

- Demographic pressure increases demand: -2 pts (Clinical)
- Hospital financial deterioration: -3 pts (Economics)
- Continued VHCURES data gaps: -1 pt (Technology)
- No equity program expansion: -1 pt (Equity)

Hospital sector outcome: 4/14 hospitals may close by 2030
Vermont total cost of care growth: ~4.5%/year

The Scenario C trajectory — hospital closures accelerating, cost growth continuing — is not a hypothetical. It is the current trajectory without intervention. Vermont has the policy infrastructure, the reform mandate, and the technical capability to achieve Scenario A outcomes. What it requires is sustained political will and operational execution.

Key Concepts Introduced in This Chapter

- Green Mountain Care Board (GMCB):** Vermont's independent health regulatory authority with oversight over hospital budgets, insurance rate approvals, and AHEAD Model performance.
- DVHA (Department of Vermont Health Access):** Vermont's Medicaid agency; co-administrator of the AHEAD Model alongside GMCB.
- Accountable Health Communities (AHC) model:** A CMMI model testing whether systematically identifying and addressing health-related social needs through community-based organizations reduces healthcare utilization and costs.
- Collaborative Care Model (CoCM):** The evidence-based integrated behavioral health model that embeds a psychiatric consultant and behavioral health care manager in primary care; CMS reimburses under CPT 99492–99494.

CHW (Community Health Worker): Trained community members who provide health education, navigation, and social needs assistance to high-need populations; one of the most cost-effective equity interventions in the evidence base.

All-payer claims database (APCD): A state-level database aggregating claims data across all payers; Vermont's APCD is VHCURES. APCDs are essential infrastructure for global budget management and population health analytics.

End of Part III
